TERUMO BLOOD AND CELL TECHNOLOGIES

AFRICA HACKATHON 2026
DELIVERED BY TALO

[WORKFLOW: E.G. EMERGENCY REQUEST, MATCHING AND DISPATCH]

[PROJECT NAME]

[TEAM NAME]

[One sentence: what your system lets which user do, e.g. "Lets a county blood bank find and dispatch the nearest suitable unit in minutes, not hours."]

01 · THE PROBLEM

[THE PROBLEM IN ONE LINE, E.G. URGENT REQUESTS GO OUT BY PHONE AND THE NEAREST UNIT IS HARD TO FIND]

WHO FEELS IT

[Name the user, e.g. "The duty officer at a county blood bank, handling 15 to 20 urgent requests a day."]

[Where they work: power, connectivity, paper registers, staff numbers.]

HOW BIG IT IS

[__ %]

[What the number measures, and its source. No source? Say how you estimated it.]

WHERE THE CURRENT WORKFLOW BREAKS

1

[Step, e.g. Hospital calls the blood bank]

[Who does it, with what tool]

→

2

[Step]

[Who does it, with what tool]

→

3

[The step that fails]

Breaks here: [what goes wrong and what it costs]

→

4

[Step]

[Who does it, with what tool]

The cost today: [time lost, units expired, patients waiting, in numbers if you have them]

03 · OUR SOLUTION

[SOLUTION NAME]: [WHAT IT DOES, IN ONE LINE]

PERSON

[Step, e.g. Request logged]

[Role and screen]

→

SYSTEM

[Step, e.g. Eligible stock found]

[What the system does]

→

AI RECOMMENDS

[Step, e.g. Best source suggested]

[What it suggests, and why]

→

PERSON APPROVES

[Step, e.g. Officer accepts]

[Decision recorded]

Built in 24 hours: [the parts that work end to end]

Mocked: [the parts shown but not built]

04 · LIVE DEMO · 5 MINUTES

[ONE USER, ONE TASK, START TO FINISH]

WE WILL SHOW

[The task, as the user, on synthetic data]

WATCH FOR

[The AI recommendation, its reasons and the approval step]

THEN

We switch the AI off and finish the task anyway

ARCHITECTURE AND WHERE IT FITS IN ABBIS

[Paste your architecture diagram here: users, front end, services, database, AI module, and the APIs between them]

STACK

[Languages, frameworks, database, hosting]

PLACE IN ABBIS

[Which ABBIS modules this extends or connects to]

STANDARDS

[e.g. HL7 FHIR, ISBT 128, or none yet]

SECURITY

[Roles and access, audit log, how data is protected]

[WHAT IT RECOMMENDS, E.G. WHICH FACILITY SHOULD SUPPLY THE REQUEST]

METHOD	[Model or method, and the data it uses]
WHY THIS?	[The plain-language reasons a user sees]
WHO DECIDES	[The role that accepts, changes or rejects, and where it is recorded]
IF IT FAILS	[How the user finishes the task with the AI off, wrong or slow]

DOES IT BEAT A BASELINE?

[Baseline, e.g. last month's average] [___]

Our model [___]

[Error measure and test data, e.g. mean absolute error on held-out synthetic weeks]

07 · BUILT FOR REAL CONDITIONS

ONE SYSTEM, MANY BLOOD BANKS

POOR CONNECTIVITY

[What works offline, and how it syncs without duplicates]

PAPER AND SPREADSHEETS

[How existing records get into the system]

ANOTHER COUNTRY

[What changes through settings: language, branding, national rules]

TRACEABILITY

[How every unit keeps one ID, and every change records who, when and why]

FROM PROTOTYPE TO A CONTROLLED PILOT

NOW

Prototype

[What works on synthetic data today]

[__ MONTHS]

MVP

[What must be built, with whom, at what indicative cost]

[__ MONTHS]

Controlled pilot

[Type of site, users, and how you measure success]

Top risks	How we would handle it
[Risk 1, e.g. data access approvals]	[Mitigation]
[Risk 2]	[Mitigation]
[Risk 3]	[Mitigation]

09 · THE TEAM

WHO BUILT IT, AND WHAT WE NEED NEXT

[Name] **[Role]** [What they built]	**[Name]** **[Role]** [What they built]	**[Name]** **[Role]** [What they built]	**[Name]** **[Role]** [What they built]	**[Name]** **[Role]** [What they built]

WHAT WE NEED TO TAKE THIS FURTHER

[One clear ask, e.g. "A pilot site and access to anonymised stock data from one county blood bank."]